01 Area map - where can our team help today?

Team leader / daily planning | Map, assessed response needs and uncovered contribution.

ReachPoint Fictional Harbor locality **SYNTHETIC FIXTURE** **Team leader / T-R relief** Clock 10:00

Area map

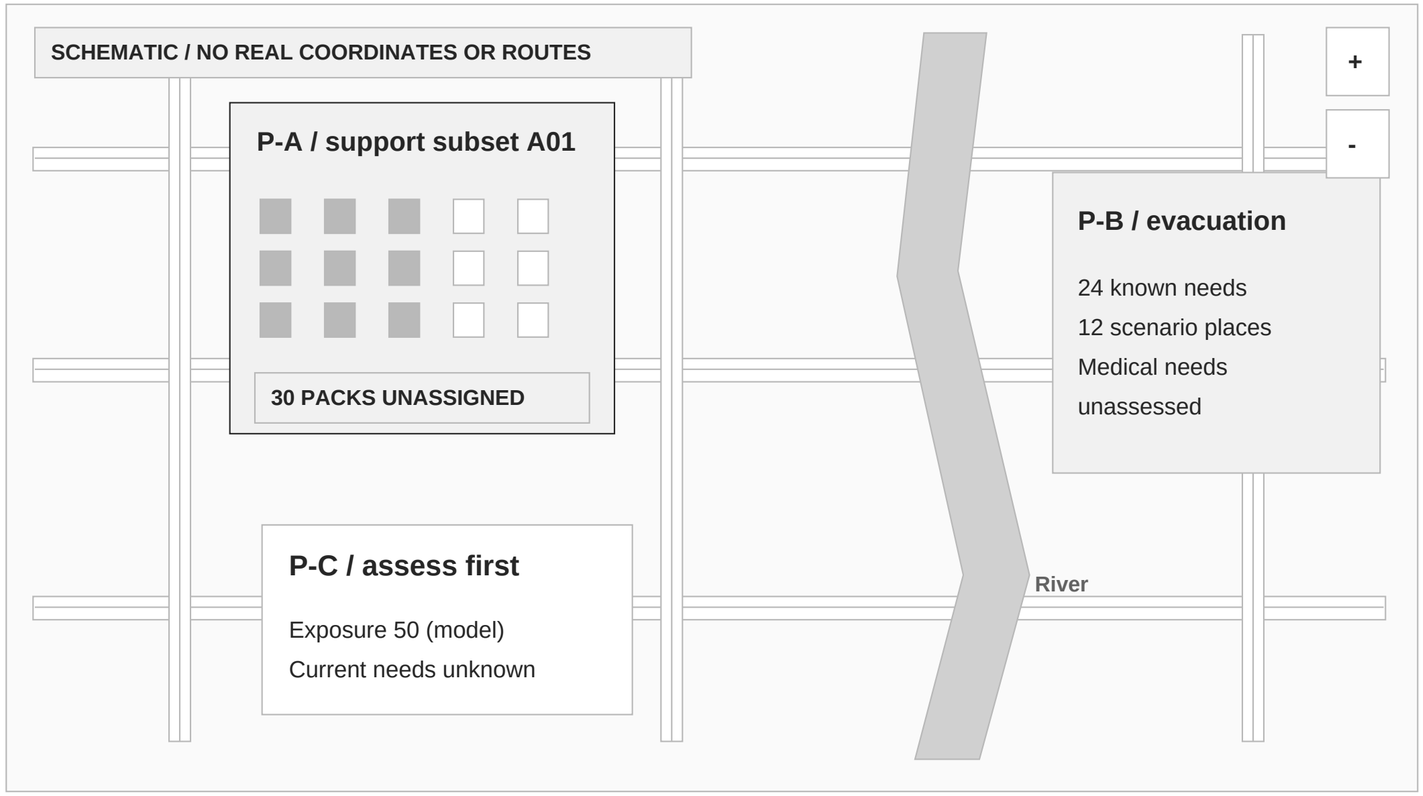
My missions

Plan today

T-R: RELIEF | 4 PEOPLE | 80 PACKS IN STOCK | PAYLOAD CHECK PENDING

[x] Flood evidence [x] Area needs [x] Existing plans [ ] Team position reports

Map [selected] / List | Extent observed 09:00 | Depth reports separate



Selected: P-A

SUPPORT IN PLACE / ASSESSED SUBSET A01

Assessed target today: 80 people
Exposure model: 100 / year 2025
Current occupancy outside A01: Unknown

A01 depth reports: 0.4-0.8 m
Synthetic ground-relative observations
Observed 09:00 / no area-wide depth
Mode basis: assessment AT-A at 09:30

60 packs remain undelivered.
30 planned elsewhere; 30 unassigned.
Not a blanket household safety label.

Plan a 30-pack relief trip

Sources
Fixture only
No live feeds

Other attention	Why it stays visible	Relevant contribution
P-B / evacuation	Known gap / medical needs unassessed	Coordinator / qualified teams
P-C / no current assessment	No report is not no need	Scoped assessment / local contact

Selection opens the same area in map, list and planner.

Building symbols and depth/population records are fixtures, not satellite inference or current observations.

02 Mission planner - what should we bring?

Team leader / P-A | Prepare a specific contribution and expose checks before commitment.

ReachPoint Fictional Harbor locality **SYNTHETIC FIXTURE** **Team leader / T-R relief** Clock 10:05

Area map

My missions

Sources
Fixture only
No live feeds

P-A / daily relief trip

DRAFT / NOT COMMITTED

Area need and coverage

Scope: assessed subset A01
Period: today
Target: 80 people
Allowance: 1 pack/person/day
Scenario allowance only

Required: 80 packs
Received / usable: 20
Remaining: 60
Accepted other plan: 30
Unassigned: 30

Scope/overlap checked in fixture.
A commitment is not a delivery.

Our proposed contribution

Mission type Relief supplies / A01

Food packs **30** Stock 80 / proposed load 30

Planned window Proposed 13:00-14:00 / requires acceptance

After this proposal: 30 still unassigned. After acceptance: 0 unassigned; 60 still undelivered.

Readiness and extra capabilities

Relief capability: declared/reviewed fixture / T-R
Payload/access/staging checks: pending / owner coordinator
Medical requirements: no reviewed request for A01 in fixture
New medical reports go to the desk; absence is not established

Submit draft for coordinator review **Inspect overlap / compare another area**

The draft does not reserve resources, assign a team or confirm arrival.

Coordinator review / team acceptance establishes planned coverage; receipt still requires its own evidence.

03 Emergency desk - validate and allocate

Coordinator / C-01 | Source-linked facts, qualified requirements and suitable-team options.

ReachPoint Fictional Harbor locality **SYNTHETIC FIXTURE** **Coordinator** Clock 10:18

Area map

Missions

Emergency desk

Sources
Fixture only
No live feeds

C-01 / reported urgent assistance

REPORT REVIEWED / NO TEAM ACCEPTED

Incoming reports

CALL-01 / C-01

10:10 / location clarified

P-C / assessment gap

No current scoped observation

Unlocated report

Keep visible / clarify landmark

Interrupted calls and unknown locations remain review work.

Evidence and review

Labeled translation:
Our household is at the community building now.
There are three of us; one needs medical help.
The neighbor stayed.

Original Thai retained in fixture.
Current location: L3 / move time unknown
Planned pickup: no accepted plan yet
Shared phone does not identify neighbor

3 PEOPLE ALREADY INCLUDED IN P-B 24

Q-01 / qualified-review fixture:
Urgent medical response capability.
Explicitly synthetic; no real clinical act.
Attribution/clinical state separate.

Inspect event decisions / Q-01

Response options

T-R relief: capability mismatch
T-S rescue: available in fixture
T-M medical: available in fixture
No automatic closest-team dispatch

Draft transport: 3 people / L3
Medical action: separate T-M offer
OP-01 fictional checks: resource,
access and recipient scope reviewed
Team acceptance is still pending

Offer scoped task to T-S

Offer medical action to T-M

No arrival window becomes confirmed
until the relevant team accepts.

Operational report review is distinct from qualified urgency/clinical assessment.

Received calls / model proposals / clinical fixtures are design states; no telephony or medical service is running.

04 Team mission - prepare against the current brief

Team lead / T-S | Own-task version, preparation, progress and reported timing.

ReachPoint Fictional Harbor locality **SYNTHETIC FIXTURE** **Team leader / T-S rescue** Clock 10:20

Area map

My missions

Sources
Fixture only
No live feeds

M-01 / C-01 evacuation assistance

ACCEPTED / BRIEF v1 ACKNOWLEDGED

Brief v1 / accepted 10:20

Scope: three people at L3 landmark

Subset of reviewed P-B demand, not extra people

Pickup L3 / receiving point D1

Window: 13:00-14:00, team-provided

Progress: Preparing / not en route

Resource S1 / reviewed scenario capacity

Access/recipient checks: OP-01 fixture

Medical action offered to T-M; not accepted

No route or clinical care inferred

Report progress / estimated window

Report blocked / unavailable resource

A new brief requires a new acknowledgment.

Preparation and communication

[x] Read acknowledged brief v1

[ ] Recheck assigned resource/capacity before departure

[ ] Check equipment per team-authorized plan

[ ] Confirm staging/recipient coordination

[ ] Report changes before departure

Resident information linked to this task

Evacuation: T-S accepted / preparing

Expected window: 13:00-14:00

Last update: 10:20

Medical: no accepted team/window yet

Submit own-team update for review

Only necessary assigned details. Report submitted is not an approved outcome.

Preparation / en route / team completion are distinct from receipt, handover and care.

Illustrative locations are named references; actual team position is not plotted or implied.

05 Resident - request help and know what is confirmed

Own request / mobile | Request and accepted-status states, not a public household map.

Request assistance

SYNTHETIC / PROCESSING PERMISSION

Tell us your location and what help you need. A coordinator will review your report.

Current landmark

Community building / L3

People needing help

3 / household report

Requested assistance

Evacuation + medical help

Submit request

Report received does not mean a team has been assigned.
Phone/proxy intake follows the same review path.

Your help status / C-01

OWN REQUEST / UPDATED 10:20

Evacuation assistance confirmed

Expected arrival window

Today, 13:00-14:00

Team T-S accepted / Preparing
Window provided by team at 10:20
Not a guaranteed arrival time

Medical assistance

Awaiting team acceptance.
Arrival time is not confirmed.

My location / situation changed

The team hasn't arrived

These reports return to the coordinator.
A relief trip alone does not confirm your inclusion.

Before assignment, arrival is unknown. Each type of assistance has its own status and window.

06 When a plan changes - keep the resident informed

Coordinator and resident / 10:30 | Resource failure, held mission and withdrawn window.

ReachPoint | Fictional Harbor locality | **SYNTHETIC FIXTURE** | **Coordinator** | Clock 10:30

Area map

Missions

Emergency desk

Sources
Fixture only
No live feeds

M-01 / S1 unavailable

HELD / NEEDS REASSESSMENT

Reassessment and current brief

TEAM-01 / S1 unavailable at 10:30
Linked M-01 transport prerequisite invalid
P-A relief mission unaffected
Replacement: Unknown / owner coordinator

Reviewed brief v2: Hold; replacement pending.
Saved 10:30 / T-S acknowledgment pending
Historical v1 acknowledgment retained
Underlying progress: Preparing / now held

Verify replacement / revise plan

Inspect team report / dependency

No replacement is offered as ready without checks.
Need remains open; task has an owner.

Resident C-01 / updated 10:30

EVACUATION DELAYED / NEW TIME UNKNOWN

Your assigned transport is unavailable. The coordinator is arranging a replacement.

Previous window 13:00-14:00 withdrawn.
New arrival time: not confirmed
Next status update: 11:00, desk commitment
An update time is not an arrival time.

Medical assistance remains awaiting acceptance.
No medical arrival window is invented.

Update my location / situation

Old windows remain in history, but cannot masquerade as the current estimate.

Status view is the v1 channel; outbound SMS/callback/push and real message delivery are not implemented.

07 Field discovery and incomplete intake - keep unknowns visible

Emergency desk supporting states | These are distinct review examples, not concurrent accepted missions.

ReachPoint Fictional Harbor locality **SYNTHETIC FIXTURE** **Coordinator** Clock 10:40

Area map

Missions

Emergency desk

Sources
Fixture only
No live feeds

Review field / incomplete reports

FIELD-PC / scoped P-C observation

PROPOSED EVENTS / NOT YET APPROVED

Local observer reports one household needing supplies in a single building subset C01. The rest was not visited.

Observer/time: synthetic field fixture / 10:35
Provisional association: H-PC
Need: assistance reported / quantity unknown
Medical request: not reported, not proven absent

Review scoped coverage separately from need.
Do not clear all P-C or manufacture household totals.

Review each event / assign follow-up

Approved scoped discovery leaves unvisited area unknown.

Interrupted emergency call

INCOMPLETE / REPORTED URGENCY VISIBLE

Permission received; call disconnected before the landmark was clarified. Received content remains an incomplete report.

Location: Unknown / no fabricated pin
Identity: unresolved / shared phone not identity
Human review owner: coordinator
Team assignment / arrival: not confirmed

Assign review / clarification action

Missing fields do not hide suspected danger.
No completed intake or rescue promise is inferred.
Real escalation protocol remains an operational gate.

Discovery reaches beyond incoming household requests; coverage remains scoped and dated.

Recovery preserves received evidence and uncertainty; no actual field visit, call or clinical decision occurred.